



House of Representatives

General Assembly

File No. 403

February Session, 2026

Substitute House Bill No. 5040

House of Representatives, April 7, 2026

The Committee on Human Services reported through REP. GILCHREST of the 18th Dist., Chairperson of the Committee on the part of the House, that the substitute bill ought to pass.

AN ACT IMPLEMENTING THE GOVERNOR'S BUDGET RECOMMENDATIONS FOR HEALTH AND HUMAN SERVICES.

Be it enacted by the Senate and House of Representatives in General Assembly convened:

1 Section 1. Subsection (b) of section 17b-191 of the 2026 supplement to
2 the general statutes is repealed and the following is substituted in lieu
3 thereof (*Effective July 1, 2026*):

4 (b) The state-administered general assistance program shall provide
5 cash assistance of [(1)] two hundred sixty-nine dollars per month [for an
6 unemployable person upon determination of such person's
7 unemployability; (2) two hundred dollars per month for a transitional
8 person who is required to pay for shelter; and (3) fifty dollars per month
9 for a transitional person who is not required to pay for shelter] to
10 persons eligible for the program. The standard of assistance paid for
11 individuals residing in rated boarding facilities shall remain at the level
12 in effect on August 31, 2003. No person shall be eligible for cash
13 assistance under the program if eligible for cash assistance under any
14 other state or federal cash assistance program. The standards of

15 assistance set forth in this subsection shall be subject to annual increases,
16 as described in subsection (b) of section 17b-104.

17 Sec. 2. Subsection (f) of section 17b-274d of the general statutes is
18 repealed and the following is substituted in lieu thereof (*Effective July 1,*
19 *2026*):

20 (f) Nonpreferred drugs in the classes of drugs included on the
21 preferred drug lists shall be subject to prior authorization. Prior
22 authorization is not required for any mental-health-related drug that
23 has been filled or refilled, in any dosage, at least one time in the one-
24 year period prior to the date the individual presents a prescription for
25 the drug at a pharmacy. If prior authorization is granted for a drug not
26 included on a preferred drug list, the authorization shall be valid for one
27 year from the date the prescription is first filled. [Antiretroviral classes
28 of drugs shall not be included on the preferred drug lists.]

29 Sec. 3. (NEW) (*Effective July 1, 2026*) (a) The Commissioner of Social
30 Services may periodically review available data on the clinical
31 effectiveness of outpatient prescription drugs covered under the
32 Medicaid program that are projected to exceed (1) a net cost per
33 consumer, after factoring in rebates, of twenty-five thousand dollars per
34 year, or (2) an annual aggregate cost, after factoring in rebates to the
35 medical assistance program, of ten million dollars. The commissioner
36 may, within available appropriations, contract with a third party to
37 conduct a comparative effectiveness review of any such outpatient
38 prescription drug. For purposes of this section, "rebate" means an
39 amount sent to the state by a prescription drug manufacturer to offset
40 the cost of outpatient prescription drugs covered by the Medicaid
41 program.

42 (b) Any such comparative effectiveness review shall include, but
43 need not be limited to: (1) Clinical efficacy and outcomes; (2)
44 information relating to the pricing of the outpatient prescription drug,
45 including, but not limited to, information relating to prices paid by other
46 states or developed nations; (3) such drug's net price to the Medicaid
47 program as compared to its therapeutic benefits, including, but not

48 limited to, the seriousness and prevalence of the disease or condition
49 that is treated by the drug; (4) the extent of utilization of such drug; (5)
50 the likelihood that the use of such drug will reduce the need for other
51 medical care; (6) the number of manufacturers that produce such drug;
52 and (7) whether there are pharmaceutical equivalents of such drug.

53 (c) The Commissioner of Social Service may make public and share
54 the results of any comparative effectiveness review with any entity,
55 including any multistate prescription drug purchasing collaborative in
56 which the state is a participating member, in order to negotiate
57 additional supplemental rebate agreements beyond any rebates
58 required under federal law.

This act shall take effect as follows and shall amend the following sections:		
Section 1	July 1, 2026	17b-191(b)
Sec. 2	July 1, 2026	17b-274d(f)
Sec. 3	July 1, 2026	New section

Section 1	July 1, 2026	17b-191(b)
Sec. 2	July 1, 2026	17b-274d(f)
Sec. 3	July 1, 2026	New section

Statement of Legislative Commissioners:

In Section 3(a)(2) and 3(b), references to "prescription drugs" were changed to "outpatient prescription drugs" or "such drug" for consistency, and the provisions of Section 3(c) were rewritten for clarity.

HS *Joint Favorable Subst.*

The following Fiscal Impact Statement and Bill Analysis are prepared for the benefit of the members of the General Assembly, solely for purposes of information, summarization and explanation and do not represent the intent of the General Assembly or either chamber thereof for any purpose. In general, fiscal impacts are based upon a variety of informational sources, including the analyst's professional knowledge. Whenever applicable, agency data is consulted as part of the analysis, however final products do not necessarily reflect an assessment from any specific department.

OFA Fiscal Note

State Impact:

Agency Affected	Fund-Effect	FY 27 \$	FY 28 \$
Social Services, Dept.	GF - Net Savings	(894,000)	(1,142,000)

Note: GF=General Fund

Municipal Impact: None

Explanation

The bill results in net savings to the Department of Social Services of \$894,000 in FY 27 and \$1,142,000 in FY 28.

Section 1 results in a cost of \$56,000 in FY 27 and \$58,000 in FY 28 associated with eliminating a reduced monthly benefit for transitional individuals under the State Administered General Assistance (SAGA) program.

Section 2 results in state Medicaid savings of \$950,000 in FY 27 and \$1.2 million in FY 28 due to adding antiretroviral drugs to the Preferred Drug List (PDL), resulting in savings associated with supplemental rebates.

The Out Years

The annualized ongoing fiscal impact identified above would continue into the future subject to service utilization and supplemental rebates.

OLR Bill Analysis**sHB 5040*****AN ACT IMPLEMENTING THE GOVERNOR'S BUDGET
RECOMMENDATIONS FOR HEALTH AND HUMAN SERVICES.*****SUMMARY**

This bill makes various unrelated changes to human services-related statutes. Specifically, the bill:

1. sets a standardized \$269 monthly cash benefit for State Administered General Assistance (SAGA) beneficiaries, thereby eliminating current law's payment reduction for certain transitional people (§ 1);
2. adds antiretroviral drugs to the Medicaid preferred drug lists, allowing the state to negotiate supplemental rebates with drug manufacturers for these drugs (§ 2); and
3. authorizes the Department of Social Services (DSS) commissioner to periodically review available data on the clinical effectiveness of certain Medicaid-covered outpatient prescription drugs (§ 3).

EFFECTIVE DATE: July 1, 2026

§ 1 — SAGA BENEFITS

The bill sets a standardized \$269 monthly cash benefit for SAGA beneficiaries (subject to future cost of living adjustments), and in doing so, eliminates current law's temporary payment reduction for certain transitional people.

By law, SAGA generally provides cash assistance to single or married childless people who have very low incomes, do not qualify for other cash assistance programs, and are considered "transitional" or "unemployable." Under current law, monthly benefits are \$262 for

unemployable individuals and transitional recipients who must pay for their shelter and \$66 for transitional recipients who do not pay for shelter (these benefit amounts account for annual cost of living adjustments). For the latter, in practice DSS sends the transitional person's medical information to an independent reviewer, and if the person is deemed medically unemployable, the person's monthly benefit retroactively increases to \$262.

§ 2 — MEDICAID PREFERRED DRUG LIST AND ANTIRETROVIRAL DRUGS

The bill adds antiretroviral drugs to the Medicaid preferred drug lists, and in doing so, allows the state to negotiate supplemental rebates with drug manufacturers for them. (This group of medications is used to prevent and treat HIV.)

By law, DSS, in consultation with the Medicaid Pharmaceutical and Therapeutics Committee, may establish a preferred drug list (formulary) that limits the number of drugs that may be dispensed under the Medicaid program. Nonpreferred drugs in the drug classes included on the preferred drug list are subject to prior authorization. Under current law, classes of antiretroviral drugs are exempt from prior authorization requirements and cannot be included on preferred drug lists.

§ 3 — PERIODIC REVIEW OF MEDICAID PRESCRIPTION DRUGS

The bill authorizes the DSS commissioner to periodically review available data on the clinical effectiveness of Medicaid-covered outpatient prescription drugs that are projected to exceed, after factoring in rebates, (1) a per-consumer net cost of \$25,000 per year or (2) an aggregate annual cost to the program of \$10 million.

Under the bill, the commissioner may, within available appropriations, contract with a third party to conduct a comparative effectiveness review of any of these prescription drugs. At a minimum, this review must include the following:

1. clinical efficacy and outcomes;

2. pricing-related information, such as prices paid by other states or developed nations;
3. the prescription drug's net price to the program compared to its therapeutic benefits, such as the seriousness and prevalence of the disease or condition the drug is treating;
4. the extent of the prescription drug's use and the likelihood that its use will reduce the need for other medical care;
5. how many manufacturers produce it; and
6. whether there are pharmaceutical equivalents.

Under the bill, the results of the review may be made public and shared with any entity or multi-state prescription drug purchasing collaborative that Connecticut is a member of, to help negotiate additional supplemental rebate agreements beyond those required under federal law.

COMMITTEE ACTION

Human Services Committee

Joint Favorable Substitute

Yea 23 Nay 0 (03/19/2026)